

D6 – Office Visit Note

P.A. Kim, MD | Family Medicine | 18 Mar 2008

Patient: Caduceus, Opus J. MRN: 004-7703 DOB: 14 Mar 1956 Allergies: NKDA

OFFICE VISIT NOTE

Patient: Opus J. Caduceus | MRN: 004-7703

Date: 03/18/2008 | Provider: P.A. Kim, MD

Visit Type: New Patient

Practice: Oak Harbor Family Medicine (TRICARE), Oak Harbor WA

CHIEF COMPLAINT: New patient establishment for chronic condition management.

HISTORY OF PRESENT ILLNESS:

51-year-old active duty USN service member, TRICARE-covered, self-referred for chronic condition management. Brings outside medical records from NAVCENT and prior MTF deployment encounters. Several chronic problems on long-term management at outside facilities; no acute concerns today. Patient interested in establishing longitudinal primary care given non-deploying status and stable Oak Harbor billet.

CURRENT MEDICATIONS:

- Omeprazole 20 mg PO daily (per outside records, started ~2005)
- Flunisolide nasal spray PRN (per outside MTF records)
- Ibuprofen PRN (patient self-treatment for bilateral knee pain)

PHYSICAL EXAMINATION:

Vitals: HR 78 | BP 128/78 | Wt 88 kg

General: Active duty male in no acute distress. Conversational.

HEENT: Mildly boggy nasal turbinates bilaterally. No acute sinus tenderness.

CV: Regular rate and rhythm. No murmurs.

Lungs: Clear bilaterally.

MSK: Mild bilateral knee tenderness with palpation, R > L. No effusion, no warmth, no erythema. Range of motion preserved.

ASSESSMENT & PLAN:

1. GERD / epigastric symptoms - chronic, on omeprazole. Partial sx control with occasional breakthrough heartburn. No dysphagia, no hematemesis. Increase omeprazole to 40 mg PO daily. GI follow-up discussed if persistent symptoms.
2. Bilateral knee pain - chronic, R > L. Self-treated with ibuprofen. No acute swelling. Continue self-treatment as needed. Orthopedic referral PRN if worsening.
3. Chronic rhinosinusitis - stable on intermittent intranasal steroid. Continue flunisolide nasal spray PRN.

Labs/Orders: CBC, CMP, HbA1c, lipid panel, TSH, UA

Follow-up: 3 months for chronic condition management and lab review.

Electronically signed by: P.A. Kim, MD

Date: 03/18/2008 14:15